

SPASIL TABLET

CONTENT

Diphenoxylate hydrochloride	2.5 mg
Atropine sulphate	0.025 mg

DESCRIPTION

White round biconvex tablet embossed with 'SPS' on one side and with a bisect on the other side.

INDICATION

Spasil tablet is used in the symptomatic treatment of acute and chronic diarrhoea. In patients with colostomies or ileostomies, **Spasil** tablet may be used to reduce the frequency and fluidity of the stools.

RECOMMENDED DOSAGE

Adults

Take 1 to 2 tablets three or four times a day or as directed by the physician. Do not exceed 8 tablets in 24 hours.

Stop treatment when bowel movements return to normal. Consult doctor if diarrhoea is bloody or persists for more than 3 days.

Children

6 to 8 years of age:	1 tablet 3 times daily
9 to 12 years of age:	1 tablet 4 times daily

For oral administration.

Not recommended for children under 6 years of age.

CONTRAINDICATIONS

Patients with known hypersensitivity to diphenoxylate hydrochloride or atropine.

In patients with jaundice, intestinal obstruction, acute ulcerative colitis, in the treatment of diarrhoea associated with pseudomembranous enterocolitis, and in patients with a raised intracranial pressure, and patients with head injury.

Contraindicated in myasthenia gravis, pyloric stenosis, paralytic ileus and prostatic enlargement.

WARNING AND PRECAUTIONS

Not recommended for children under 6 years of age.

In the presence of severe dehydration and electrolyte imbalance, corrective therapy must be initiated before **Spasil** tablet is given. Appropriate fluid and electrolyte therapy should be given to protect against dehydration in all cases of diarrhoea. Oral rehydration therapy, which is the use of appropriate fluids including oral rehydration salt, remains the most effective treatment for dehydration due to diarrhoea. The intake of as much of these fluids as possible is therefore imperative. Drug-induced inhibition of peristalsis may result in fluid detention in the intestine, which may aggravate and mask dehydration and depletion of electrolytes, especially in young children. If severe dehydration of electrolyte imbalance is present, **Spasil** should be withheld until appropriate corrective therapy has been initiated.

Spasil should be used with extreme caution in patients with advanced hepatorenal disease, and in patients with abnormal liver function, since hepatic coma may be precipitated.

Because a subtherapeutic dose of atropine is added to the medicine, atropine effects, may occur in susceptible individuals or in overdosage. Individuals with Down's syndrome appear to have increased susceptibility to the action of atropine.

Spasil may be habit forming.

Effects on ability to drive and use machines

Some of the undesirable effects such as sedation, drowsiness or dizziness may affect the ability to drive or operate machines. If affected, patients should be advised not to drive or operate machinery.

INTERACTIONS

- Antagonises the effects of Domperidone, Metoclopramide, Bethanecol/Carbachol, Cisapride, Galantamine, Neostigmine/Pyridostigmine and Pilcarpine.
- Antimuscarinic side effects of **Spasil** increased by Amantadine, Antihistamine (sedative and non-sedative), Clozapine, Disopyramide, Fluspiriline, Loxapine, MAOIs, Nefopam, Olanzapine, Phenothiazines, Quantiapine, Remoxipride, Terfenadine, Tricyclic antidepressants and Zotepine.
- Dry mouth prevents the dissolution of sublingual nitrate tablets, such as glyceryl trinitrate.
- Effect of **Spasil** is antagonized by Dompezil.
- Effect of **Spasil** is increased by Memantine.
- May reduce the plasma levels of Levodopa (in combination and sole products).
- Reduced the absorption of Ketoconazole.

PREGNANCY AND LACTATION

There are no adequate and well-controlled studies in pregnant women. **Spasil** should only be used if the potential benefit justified the risk to the mother and foetus. Atropine sulphate can cross the placenta.

Diphenoxylate hydrochloride and atropine sulphate may be excreted in human milk. If a nursing mother is taking **Spasil**, the infant may exhibit some effects of the drug.

SIDE EFFECTS

Central Nervous System

Euphoria, confusion, dizziness, restlessness, malaise, lethargy, sedation, somnolence, hallucinations, headache, fever, depression, insomnia and numbness of the extremities.

Allergic reactions

Anaphylaxis, angioedema, urticarial and pruritus.

Gastrointestinal system

Paralytic ileus, toxic megacolon, nausea, vomiting, anorexia, abdominal distension, constipation and swelling of the gum.

Eye disorders

Dilation of pupils with loss of accommodation, photophobia and very rarely angle closure glaucoma.

Atropine side effects of **Spasil** include flushing, dryness of the skin and mucous membranes (including dry mouth), cardiac irregularities such as arrhythmias, bradycardia and palpitations, increased intra-ocular pressure, difficulty in swallowing, hyperthermia, urinary retention and difficulty in micturition and respiratory depression in children.

SYMPTOMS AND TREATMENT OF OVERDOSE

Overdose may produce narcosis with respiratory depression, atropine poisoning, or both, particularly in children. Symptoms of overdose include dryness of the skin and mucous membranes, flushing, hypothermia and tachycardia, nystagmus, pinpoint pupils, hypotonic reflexes, lethargy, coma and severe respiratory depression. The onset of symptoms of overdose may be considerably delayed and respiratory depression may not become evident until as late as 12–30 hours after ingestion, and may occur in spite of initial response to narcotic antagonists. Patient should be hospitalized and closely monitored for 48 hours. Gastric lavage and induced emesis should be performed immediately if the patient is not comatose.

If respiratory depression develops, naloxone should be administered. The duration of action of naloxone hydrochloride is considerably shorter than that of diphenoxylate hydrochloride and repeated injections of the antidote may be required. Establishment of a patient airway and artificial ventilation may be needed.

PHARMACODYNAMICS

The active ingredient diphenoxylate hydrochloride is a synthetic opioid derivative with selective effects on gastrointestinal smooth

muscle. It is essentially devoid of “morphine type subjective effects” at therapeutic doses.

Atropine sulphate is a tertiary amine antimuscarinic alkaloid which has inhibitory effect on gastrointestinal motility and antispasmodic actions on smooth muscle.

PHARMACOKINETICS

Diphenoxylate hydrochloride is well absorbed from the gastrointestinal tract and extensively metabolized in the liver to diphenoxylate acid (dephenoxin), and hydroxyphenoxylate acid. It is excreted mainly as metabolites in the urine and bile.

Atropine is readily absorbed from the gastrointestinal tract and mucous membranes. It is metabolized in the liver and is excreted in the urine as unchanged drug and metabolites.

LIST OF EXCIPIENTS

Magnesium stearate, Colloidal silicon dioxide, Povidone K-90, Talcum, Cornstarch, Lactose monohydrate

PRESENTATION

Aluminium foil strip of 10 tablets. Box of 10 x 10's and 100 x 10's.

STORAGE CONDITIONS & PHARMACEUTICAL PRECAUTIONS

Store in a cool dry place, below 30°C.

Keep the tablets in the original packaging to protect from moisture and light.

Keep out of reach of children.

Jauhi daripada kanak-kanak.

SHELF LIFE: Please refer to outer box label.

MANUFACTURED BY:



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